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DEPARTMENT OF HEALTH AND HUMAN SERVICES
Centers for Medicare & Medicaid Services
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Request for Information; Health Technology Ecosystem
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Request for Information; Health Technology Ecosystem

Patient.com responses to questions in bold print.

A. *Patients and Caregivers*

1. Patient Needs

PC-1. What health management or care navigation apps would help you understand and manage your (or your loved ones) health needs, as well as the actions you should take?

Answer:

At Patient.com, we believe the key to solving healthcare's inefficiencies isn't just technology—it's empathy. And that begins by naming what so many Americans experience: Patient Flustration™. It's the compound feeling of being both flustered and frustrated by a system that claims to be digital, yet still buries people in paperwork, portals, passwords, and phone trees.

This emotional friction leads to real-world consequences—collection issues, delayed communication, missed opportunities for efficiency, and unnecessary stress for patients and caregivers alike. It also places avoidable strain on the entire healthcare ecosystem. Patient.com is on a mission to end Patient Flustration by putting patients back in control—because when you empower the patient, you transform the system.

a. What are the top things you would like to be able to do for your or your loved ones' health that can be enabled by digital health products?

Answer:

Track health history and needs for the entire family and as a non-family caregiver. One app to access a complete immunization and clinical history across EMR systems,

states and without having to remember login credentials for all and old portals.

b. If you had a personal assistant to support your health needs, what are the top things you would ask them to help with? In your response, please consider tasks that could be supported or facilitated by software solutions in the future.

Answer:

A personal assistant would help most with appointment setting and administrative task management. Current applications for setting appointments can be incredibly laborious with multiple steps. AI and FHIR resources for appointment scheduling would help consumers be able to have fewer steps to complete an appointment. Then, completing all the pre-appointment consents and forms would be far easier by having standard, shareable consents and use of FHIR resources.

PC-2. Do you have easy access to your own and all your loved ones' health information in one location (for example, in a single patient portal or another software system)?

Answer:

Not in one location, but on multiple portals. Family sharing is sometimes possible and there is a range of easy to difficult set-up. If a family member has separate insurance, separate portal access is required after access is granted by the owner. Access to all date through multiple portals remains a very disjointed process that needs to be updated often.

a. If so, what are some examples of benefits it has provided?

Answer:

Having access to a family member's portal makes it easier for a caregiver to track clinical care, including labs and immunizations.

b. If not, in what contexts or for what workflows would it be most valuable to use one portal or system to access all such health information?

Answer:

Still need better access to data, not through portals and across health systems, EMRs and states.

c. Were there particular data types, such as x-rays or specific test results, that were unavailable? What are the obstacles to accessing your own or your loved ones' complete health information

electronically and using it for managing health conditions or finding the best care (for example, limitations in functionality, user friendliness, or access to basic technology infrastructure)?

Answer:

Radiology and Pathology results can still prove challenging to access. For instance, mammogram results come in a mailed letter and the report appears in an app as a FHIR resource that does not appear to include the interpretation. Radiology images are so important and so burdensome to obtain and store. Nobody can manage a burned CD or DVD, and even a USB is still a physical object that needs to be transported. The industry could do better by providing access to DICOM and other imaging with efficient, short-term storage.

PC-3. Are you aware of health management, care navigation, or personal health record apps that would be useful to Medicare beneficiaries and their caregivers?

Answer:

Patient.com will be very easy for Medicare beneficiaries to use and this population is a specific focus for user experience design and testing. As noted in the Executive Summary above, we are suggesting that Medicare, Blue Button 2.0 and HETS provide bulk data access to patients without the need for individual patient and provider credentials.

PC-4. What features are missing from apps you use or that you are aware of today?

Answer: Translation of the interpretation radiology and operative reports into non-technical terms would be very helpful.

a. What apps should exist but do not yet? Why do you believe they do not exist yet?

Answer:

Health care is complex! All-in-one applications that are vendor and provider agnostic and that do more than just one service would be so helpful!

b. What set of workflows do you believe CMS is uniquely positioned to offer?

Answer:

CMS could provide more quality rating information. This should be during Medicare Advantage open enrollment and through brokers. It could also be considered with referrals/ prior authorizations for Long Term Care facilities, hospitals, and providers – groups, clinics or individuals. As noted in the Executive Summary above, we are

suggesting that Medicare, Blue Button 2.0 and HETS provide bulk data access to patients without the need for individual patient and provider credentials.

PC-5. What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?

Answer:

We believe in radical usability. CMS should highlight specific benefits for safety, efficiency and education, including translation of medical jargon into patient friendly terms and explanations.

a. What role, if any, should CMS have in reviewing or approving digital health products on the basis of their efficacy, quality or impact or both on health outcomes (not approving in the sense of a coverage determination)? What criteria should be used if there is a review process? What technology solutions, policy changes, or program design changes can increase patient and caregiver adoption of digital health products (for example, enhancements to data access, reimbursement adjustments, or new beneficiary communications)?

Answer:

CMS should allow the market to determine the best products and address issues when identified. Quality and Safety scores for digital health products would be helpful for consumers to make informed choices.

b. What changes would enable timely access to high quality CMS and provider generated data on patients?

Answer:

ANSI 5010 X12 data should be more accessible to patients. It should be available to digital health products in bulk and without provider credentials. IAS data should be more accessible. QHIN participants returning data for the required IAS exchange purpose is currently far lower than it should be.

PC-6. What features are most important to make digital health products accessible and easy to use for Medicare beneficiaries and caregivers, particularly those with limited prior experience using digital tools and services?

Answer:

Medicare beneficiaries and caregivers deserve radical usability; clear, explanatory

applications that initially present a high-level view of data and processes, that can become more robust as the user needs. Applications should be flexible and easily user-configurable, ADA-compliant and apply device operating system settings like large font or text-to-voice without the user having to set these preferences again and again.

PC-7. If CMS were to collect real-world data on digital health products' impact on health outcomes and related costs once they are released into the market, what would be the best means of doing so?

Answer:

A CAHPS question on the value of digital health products would be one direction. For specific apps' health outcomes benefits, perhaps payers and providers could be asked to report on the apps used for patient engagement and provide qualitative analysis on health outcome benefit as a starting point, that could evolve into quantitative once large volume of patient engagement in the app is realized.

2. Data Access and Integration

PC-8. In your experience, what health data is readily available and valuable to patients or their caregivers or both?

Answer:

On patient portals and some PHR apps, lab data seems to be most readily available. Sections of the medical record, such as open notes, allergies, problem lists and often diagnosis and procedure lists are also available for the connected data sources, which may not be all sources that the patient needs.

a. What data is valuable, but hard for patients and caregivers, or app developers and other technical vendors, to access for appropriate and valuable use (for example, claims data, clinical data, encounter notes, operative reports, appointment schedules, prices)?

Answer:

Claims and clinical data are relatively available, depending on the connected data sources for providers but not easily for patients.

Allow digital health products to facilitate patients' access to ASC X12 Version 5010 transaction data through HETS and other clearinghouses without the necessity of a provider's credentials within the request. The current X12 transaction set requests are

limited to providers. The existing EDI system should be accessible by digital applications on behalf of patients. This small change would unlock significant value for patients without technical changes for HETS or clearinghouses. Examples of request types that should be available to patients include claims or encounters (837), eligibility and benefit information (270) and referrals and authorizations for services (278).

Remove the HETS provider affidavit requirement for ASC X12 Version 5010 transactions so that patients may consent to allow digital health products to leverage the HETS system for benefits, claims, and other data accessed by providers.

Allow bulk ASC X12 Version 5010 transactions requested by digital products on behalf of patients without requiring provider credentials. For efficiency, digital health products should be able to obtain massive amounts of data on behalf of patients as long as identity is verified at IAL2 and consent informed, stored and auditable.

From notes and reports, the diagnosis/ problem list, assessment and plan or the interpretation of the radiology reports would be valuable subsets of data for patients, as opposed to the entire note or report.

Radiology images and videos are probably the most sought after and difficult data types to obtain. CMS could lead the way in efficient transmission and short-term cloud storage of cine and images.

b. What are specific sources, other than claims and clinical data, that would be of highest value, and why?

Answer:

Benefits information and Prior Authorization status updates would be very helpful for patients.

c. What specific opportunities and challenges exist to improve accessibility, interoperability and integration of clinical data from different sources to enable more meaningful clinical research and generation of actionable evidence?

Answer:

EMRs with an integrated portal have done very well in sharing data within their internal and integrated systems. We were very excited about the Qualified Health Information Network, but adding participants has been slower than expected. Also, very

few are exchanging for the IAS exchange purpose.

Resolve information blocking.

Allow digital health products to use the "High Confidence" Identity Assurance Level 2 (IAL2) and Authenticator Assurance Level 2 (AAL2) (NIST SP 800-63-3) processes to access Clearinghouse, Blue Button 2.0 and QHIN clinical data in multiple ways. When patient consent is informed, stored and auditable, the patient should be able to access their healthcare data upon request and without extra blocking steps.

A BAA should not be required with a Covered Entity as long as the digital health product has auditable patient consent and complies with IAL2 and AAL2 processes.

Digital Health Products should be able to use IAL2 and AAL2 service which is developed and managed internally or from any suitable vendor and not only the six certified Credential Service Providers.

Instead of requiring individual login and password credentials, as with Blue Button 2.0 and other Payer Patient Access APIs, allow bulk access to the data using demographics gathered from the IAL2 process.

Reconsider the administrative necessity of the Common Agreement for Trusted Exchange Framework. A contract in this scenario may be limiting possible networks, participants and sub-participants from joining. Decreased management of this contract process would lead to cost reduction. Multiple Rules, Requirements and Standard Operating Procedures are well established to maintain a Trusted Exchange Framework.

Encourage and incentivize participation of Covered Entities in the Trusted Exchange Framework Common Agreement (TEFCA) Qualified Health Information Network (QHIN).

Penalize TEFCA QHIN participants and subparticipants who do not return data for the currently required exchange purposes of Treatment (T-TREAT) and Individual Access Services (T-IAS). Various disincentives are currently used by CMS, ASTP and OIG which are a progression from probation and then exclusion from the QHIN, or monetary penalties such as fines or reduction of Medicare payments as in the Hospital Readmissions Reduction Program. Unfortunately, the impacts of these programs have not yet had the desired effect and need advancement.

The Recognized Coordinating Entity (RCE) manages the QHIN directory. Access to this listing remains limited to RCE and QHINs but should be made public as soon as possible and this could help pressure non-responding participants.

Create a modified version of TEFCA for Individual Access Services with terms that recognize the generation of queries with a patient's consent, invoking FTC Acti and the Health Breach Notification Ruleii obligations instead of HIPAA.

Ensure all data queried is returned without geofencing: limiting data returned within an arbitrary geographic region thought to be the most likely area within which a patient received clinical services.

Ensure the availability of complete price information in a standard data format for clinical products and services to patients prior to encounters and order fulfillment that is easy to access and comprehend. As per the Executive Order 14221, "The Secretary of the Treasury, the Secretary of Labor, and the Secretary of Health and Human Services shall take all necessary and appropriate action to rapidly implement and enforce the healthcare price transparency regulations". Similar disincentives should be invoked as mentioned above: progression from probation and then exclusion from Medicare, or monetary penalties such as fines or reduction of Medicare payments as in the Hospital Readmissions Reduction Program.

PC-9. Given that the Blue Button 2.0 API only includes basic patient demographic, Medicare coverage, and claims data (Part A, B, D), what additional CMS data sources do developers view as most valuable for inclusion in the API to enable more useful digital products for patients and caretakers?

Answer:

While clinical data could be added in the ways described above, we also suggest safety and quality data pertinent to that Medicare Beneficiary. Individual care gaps should be identified and closed. Safety and quality scores for providers in the beneficiaries' network as compared to others in the region should always be readily available, but in particular, at open enrollment and in the referral/ prior authorization process.

a. What difficulties are there in accessing or utilizing these data sources today?

Answer:

Blue Button 2.0 requires a user to sign in with the Medicare.gov credentials (log in and password) and then select the data sources and types. This is quite a bit of work that likely should be streamlined. Instead of requiring individual login and password credentials, as with Blue Button 2.0 and other Payer Patient Access APIs, allow bulk access to the data using demographics gathered from the IAL2 process.

b. What suggestions do you have to improve the Blue Button 2.0 API experience?

Answer:

Instead of requiring individual login and password credentials, as with Blue Button 2.0 and other Payer Patient Access APIs, allow bulk access to the data using demographics gathered from the IAL2 process.

c. Is there non-CMS data that should be included in the API?

Answer:

Individual care gaps and provider network safety and quality data should be included through the API.

PC-10. How is the Trusted Exchange Framework and Common Agreement™ (TEFCA™) currently helping to advance patient access to health information in the real world?

Answer:

In some cases, broader access to clinical data.

b. What changes would you suggest?

Answer:

Resolve information blocking.

Allow digital health products to use the "High Confidence" Identity Assurance Level 2 (IAL2) and Authenticator Assurance Level 2 (AAL2) (NIST SP 800-63-3) processes to access Clearinghouse, Blue Button 2.0 and QHIN clinical data in multiple ways. When patient consent is informed, stored and auditable, the patient should be able to access their healthcare data upon request and without extra blocking steps.

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Create a modified version of TEFCA for Individual Access Services with terms that recognize the generation of queries with a patient's consent, invoking FTC Actiii and the Health Breach Notification Ruleiv obligations instead of HIPAA.

Ensure all data queried is returned without geofencing: limiting data returned within an arbitrary geographic region thought to be the most likely area within which a patient received clinical services.

c. What use cases could have a significant impact if implemented through TEFCA?

Answer:

TEFCA could provide more insights regarding trends that suggest medication

seeking or fraud.

d. What standards are you aware of that are currently working well to advance access and existing exchange purposes?

Answer:

USCDI is very helpful as a standard, but we are progressing in data classes and elements too slowly. Also, advance to a universal FHIR standard for transmission of data.

f. Are there redundant standards, protocols, or channels that should be consolidated?

Answer:

Review the Standards Version Advancement Process (SVAP). Progression to higher levels of standards should be faster.

g. Are there adequate alternatives outside of TEFCA for achieving widespread patient access to their health information?

Answer:

Not easily. Alternatives outside of TEFCA to access clinical data entail connecting to as many endpoints as possible.

Allow digital health products to facilitate patients' access to ASC X12 Version 5010 transaction data through HETS and other clearinghouses without the necessity of a provider's credentials within the request. The current X12 transaction set requests are limited to providers. The existing EDI system should be accessible by digital applications on behalf of patients. This small change would unlock significant value for patients without technical changes for HETS or clearinghouses. Examples of request types that should be available to patients include claims or encounters (837), eligibility and benefit information (270) and referrals and authorizations for services (278).

Remove the HETS provider affidavit requirement for ASC X12 Version 5010 transactions so that patients may consent to allow digital health products to leverage the HETS system for benefits, claims, and other data accessed by providers.

Allow bulk ASC X12 Version 5010 transactions requested by digital products on behalf of patients without requiring provider credentials. For efficiency, digital health products should be able to obtain massive amounts of data on behalf of patients as long as identity is verified at IAL2 and consent informed, stored and auditable.

PC-11. How are health information exchanges (HIEs) currently helping to advance patient access to health information in the real world?

Answer:

HIEs allow exchange of data in local, regional and state areas. Some are very mature and robust.

a. How valuable, available, and accurate do you find the data they share to be?

Answer:

HIEs are dependent on participants' vigilance to standards and transmission expertise.

b. What changes would you suggest?

Answer:

Continue to advance standards to an end-point that all HIEs, HINs and clinical data systems can adhere to and maintain.

d. What is the ongoing role of HIEs amidst other entities facilitating data exchange and broader frameworks for data exchange (for example, vendor health information networks, TEFCA, private exchange networks, etc.)?

Answer:

HIEs are still very important for local, regional access. The QHIN may not be achievable for some provider organizations or other clinical data systems for a while.

PC-12. What are the most valuable operational health data use cases for patients and caregivers that, if addressed, would create more efficient care navigation or eliminate barriers to competition among providers or both?

a. Examples may include the following:

(1) Binding cost estimates for pre-defined periods.

Answer:

Price Transparency is so valuable that two Executive Orders have been devoted to it. As per the Executive Order 14221, "The Secretary of the Treasury, the Secretary of Labor, and the Secretary of Health and Human Services shall take all necessary and appropriate action to rapidly implement and enforce the healthcare price transparency regulations". Need to ensure the availability of complete price information in a standard

data format for clinical products and services to patients prior to encounters and order fulfillment that is easy to access and comprehend.

(2) Viewing provider schedule availability.

(3) Using third-party apps for appointment management.

(4) Accessing patient-facing quality metrics.

Answer:

Patient-facing quality metrics before services are incredibly important and difficult to access. Not having access to this data threatens patient safety and quality of service.

3. Information Blocking and Digital Identity

PC-13. How can CMS encourage patients and caregivers to submit information blocking complaints to ASTP/ONC's Information Blocking Portal? What would be the impact? Would increasing reporting of complaints advance or negatively impact data exchange?

Answer:

Access to this portal should be available in any application managing QHIN data. Currently, the portal is not very easy to access. Categories should be established that describe the reporter. Contextual consideration can be given for consumer versus health technology vendor or provider organization reports.

PC-14. Regarding digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 credentialing service providers (CSP)):

a. What are the challenges today in getting patients/caregivers to sign up and use digital identity credentials?

Answer:

The ID verification process has multiple steps which may result in fear or concern by the consumer to divulge all the information and documentation needed. Explanations should be extremely clear and educational about the benefits.

d. How would encouraging the use of CSPs improve access to health information?

Answer:

In order to improve access to health information, Digital Health Products should be able to use IAL2 and AAL2 service which is developed and managed internally or from any

suitable vendor and not only the six certified Credential Service Providers. Allow digital health products to use the "High Confidence" Identity Assurance Level 2 (IAL2) and Authenticator Assurance Level 2 (AAL2) (NIST SP 800-63-3) processes to access Clearinghouse, Blue Button 2.0 and QHIN clinical data in multiple ways. Instead of requiring individual login and password credentials, as with Blue Button 2.0 and other Payer Patient Access APIs, allow bulk access to the data using demographics gathered from the IAL2 process.

E. Technology Vendors, Data Providers, and Networks

1. Ecosystem

TD-1. What short term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

Answer:

Reduce the regulatory and cost burden to accessing data from CMS, Blue Button 2.0 and HETS. Allow digital health products to use the "High Confidence" Identity Assurance Level 2 (IAL2) and Authenticator Assurance Level 2 (AAL2) (NIST SP 800-63-3) processes to access Clearinghouse, Blue Button 2.0 and QHIN clinical data in multiple ways. When patient consent is informed, stored and auditable, the patient should be able to access their healthcare data upon request and without extra blocking steps.

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Reconsider the administrative necessity of the Common Agreement for Trusted Exchange Framework. A contract in this scenario may be limiting possible networks,

participants and sub-participants from joining. Decreased management of this contract process would lead to cost reduction. Multiple Rules, Requirements and Standard Operating Procedures are well established to maintain a Trusted Exchange Framework.

Encourage and incentivize participation of Covered Entities in the Trusted Exchange Framework Common Agreement (TEFCA) Qualified Health Information Network (QHIN). Penalize TEFCA QHIN participants and subparticipants who do not return data for the currently required exchange purposes of Treatment (T-TREAT) and Individual Access Services (T-IAS). Various disincentives are currently used by CMS, ASTP and OIG which are a progression from probation and then exclusion from the QHIN, or monetary penalties such as fines or reduction of Medicare payments as in the Hospital Readmissions Reduction Program. Unfortunately, the impacts of these programs have not yet had the desired effect and need advancement.

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Create a modified version of TEFCA for Individual Access Services with terms that recognize the generation of queries with a patient's consent, invoking FTC Actvi and the Health Breach Notification Rule obligations instead of HIPAA.

TD-2. Regarding CMS Data, to stimulate developer interest--

Answer:

Allow digital health products to facilitate patients' access to ASC X12 Version 5010 transaction data through HETS and other clearinghouses without the necessity of a provider's credentials within the request. The current X12 transaction set requests are limited to providers. The existing EDI system should be accessible by digital applications on behalf of patients. This small change would unlock significant value for patients without technical changes for HETS or clearinghouses. Examples of request types that should be available to patients include claims or encounters (837), eligibility and benefit information (270) and referrals and authorizations for services (278).

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transactions so that patients may consent to allow digital health products to leverage the HETS system for benefits, claims, and other data accessed by providers.

Allow bulk ASC X12 Version 5010 transactions requested by digital products on behalf of patients without requiring provider credentials. For efficiency, digital health products should be able to obtain massive amounts of data on behalf of patients as long as identity is verified at IAL2 and consent informed, stored and auditable.

b. What data sources are most valuable alongside the data available through the Blue Button 2.0 API?

Answer:

While clinical data through the API, we also suggest safety and quality data pertinent to that Medicare Beneficiary. Individual care gaps should be identified and closed. Safety and quality scores for providers in the beneficiaries' network as compared to others in the region should always be readily available, but in particular, at open enrollment and in the referral/ prior authorization process.

2. Digital Identity

TD-3. Regarding digital identity implementation:

b. How would requiring digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 CSPs) impact cybersecurity and data exchange?

Answer:

Digital health products should use the "High Confidence" Identity Assurance Level 2 (IAL2) and Authenticator Assurance Level 2 (AAL2) (NIST SP 800-63-3) processes to access Clearinghouse, Blue Button 2.0 and QHIN clinical data. Digital Health Products should be able to use IAL2 and AAL2 service which is developed and managed internally or from any suitable vendor and not only the six certified Credential Service Providers.

TD-5. How could a nationwide provider directory of FHIR endpoints improve access to health information for patients, providers, and payers? Who should publish such a directory, and should users bear a cost?

Answer:

We understood the Recognized Coordinating Entity and thus, ASTP was

responsible for this directory. Either one of these two entities makes the most sense. Users should not bear a cost up front but perhaps fines if information is not correct or kept up to date.

TD-7. To what degree has USCDI improved interoperability and exchange and what are its limitations?

Answer:

United States Core Data for Interoperability (USCDI) has been very helpful as a standard to use for clinical data categorization and resource or document creation.

We recommend that the Standards Version Advancement Process (SVAP) be reviewed for any concerns and if none, promote the United States Core Data for Interoperability (USCDI) to the current approved version, 5. Review and approve new versions of USCDI faster, at least quarterly. Incorporate more data classes and elements in USCDI versions.

a. Does it contain the full extent of data elements you need?

Answer:

United States Core Data for Interoperability (USCDI) does not yet contain all needed data classes and elements. Many are in Levels 0-3 awaiting approval. Patient entered data elements need to be considered more closely, although attribution to a patient with currently available data elements may be enough.

b. If not, is it because of limitations in the definition of the USCDI format or the way it is utilized?

Answer:

The limitation of the United States Core Data for Interoperability (USCDI) is that is still does not yet contain all needed data classes and elements. We recommend review and approval new versions of USCDI faster, at least quarterly and incorporating more data classes and elements in USCDI versions.

c. If so, would adding more data elements to USCDI add value or create scoping challenges? How could such challenges be addressed?

Answer:

The limitation of the United States Core Data for Interoperability (USCDI) is that is still does not yet contain all needed data classes and elements. We believe adding

more and faster will result in better standards for interoperability, data sharing and comprehension of a more complete clinical picture.

d. Given improvements in language models, would you prefer a non-proprietary but less structured format that might improve data coverage even if it requires more processing by the receiver?

Answer:

The structure of United States Core Data for Interoperability (USCDI) should be maintained even within language models.

TD-15. Regarding bulk FHIR APIs:

a. How would increased use of bulk FHIR improve use cases and data flow?

Answer:

We recommend the use of bulk FHIR to accelerate large subpopulations of data flow.

b. What are the potential disadvantages of their use?

Answer:

Some systems will take longer to be able to manage a bulk FHIR process and may need more incentive(s) and technical assistance.

TD-16. What are the tradeoffs of maintaining point-to-point models vs. shared network infrastructure?

Answer:

Point to Point models provide a more controlled and limited data sharing environment while a shared network allows for a larger system with more flexibility in pathways of data. Both are useful in the right scenario.

3. Compliance

TD-18. Information blocking:

a. Could you, as a technology vendor, provide examples for the types of practices you have experienced that may constitute information blocking. Please include both situations of non-responsiveness as well as situations that may cause a failure or unusable response?

Answer:

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Ensure all data queried is returned without geofencing: limiting data returned within an arbitrary geographic region thought to be the most likely area within which a patient received clinical services.

b. What additional policies could ASTP/ONC and CMS implement to further discourage healthcare providers from engaging in information blocking practices?

Answer:

Encourage and incentivize participation of Covered Entities in the Trusted Exchange Framework Common Agreement (TEFCA) Qualified Health Information Network (QHIN).

Penalize TEFCA QHIN participants and subparticipants who do not return data for the currently required exchange purposes of Treatment (T-TREAT) and Individual Access Services (T-IAS). Various disincentives are currently used by CMS, ASTP and OIG which are a progression from probation and then exclusion from the QHIN, or monetary penalties such as fines or reduction of Medicare payments as in the Hospital Readmissions Reduction Program. Unfortunately, the impacts of these programs have not yet had the desired effect and need advancement.

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Ensure the availability of complete price information in a standard data format for clinical products and services to patients prior to encounters and order fulfillment that is easy to access and comprehend. As per the Executive Order 14221, "The Secretary of the Treasury, the Secretary of Labor, and the Secretary of Health and Human Services shall take all necessary and appropriate action to rapidly implement and enforce the healthcare price transparency regulations". Similar disincentives should be invoked as mentioned above: progression from probation and then exclusion from Medicare, or monetary penalties such as fines or reduction of Medicare payments as in the Hospital Readmissions Reduction Program.

Endnotes

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[i] Federal Trade Commission Act. 15 U.S.C. §§ 41-58, as amended. https://www.ftc.gov/legal-library/browse/statutes/federal-trade-commission-act

[ii] Federal Trade Commission, Health Breach Notification Rule. 16 CFR Part 318. https://www.ftc.gov/legal-library/browse/rules/health-breach-notification-rule

[iii] Federal Trade Commission Act. 15 U.S.C. §§ 41-58, as amended. https://www.ftc.gov/legal-library/browse/statutes/federal-trade-commission-act

[iv] Federal Trade Commission, Health Breach Notification Rule. 16 CFR Part 318. https://www.ftc.gov/legal-library/browse/rules/health-breach-notification-rule

[v] Kantara Initiative International Trust Status List. https://kantarainitiative.org/trust-status-list/

[vi] Federal Trade Commission Act. 15 U.S.C. §§ 41-58, as amended. https://www.ftc.gov/legal-library/browse/statutes/federal-trade-commission-act

[vii] Federal Trade Commission Act. 15 U.S.C. §§ 41-58, as amended. https://www.ftc.gov/legal-library/browse/statutes/federal-trade-commission-act

[viii] Federal Trade Commission, Health Breach Notification Rule. 16 CFR Part 318. https://www.ftc.gov/legal-library/browse/rules/health-breach-notification-rule